

Your Health Story

A simple summary of where you've been & where you are

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Mr. Verma, this is a plain-language summary of **everything we know about your case** — written for you, not for the doctor.

You saw Dr. Sharma on 23 June 2025 with neck pain, lower back pain, and tingling in your arms and legs that have been bothering you for about a year. Over two days you had an MRI, a nerve test, and a panel of blood tests.

The results show two separate things happening at once: one is mechanical — wear and pressure on the nerves in your spine. The other is a separate issue with the nerves themselves, likely from something in your body chemistry. Knowing it's two things, not one, is what makes the plan ahead complete.

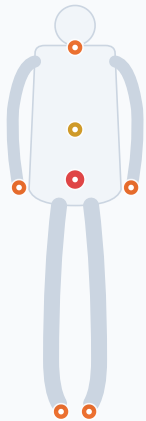
YOUR JOURNEY SO FAR

OVER THE LAST DECADE



WHAT'S HAPPENING IN YOUR BODY

IN PLAIN TERMS



Your neck (where you had surgery in 2014)

The fusion has held up. But the spine around the fused area has aged over 11 years, and some new wear has formed at the levels just above and below. There is a small change in your spinal cord at the C5 level that we want to keep an eye on — nothing urgent, but worth a follow-up scan if symptoms change.

Your lower back (the main pain source)

There is a tight spot at one specific level (L4-L5) where the channel that nerves travel through has narrowed significantly. Both of the nerves on either side at that level are being pressed on. This is what's causing most of the low back pain and the tingling in your legs.

Your hands & feet (the tingling)

The nerve test shows that the nerves themselves — not the connection at the spine — are not signalling as fast as they should. This is a separate issue, often caused by low vitamin B12, blood sugar, or thyroid imbalance. That's why we ordered the blood tests — they will tell us which one to treat.

What's next

Your medicines, what to expect, and when to call us

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YOUR MEDICINES

Esogrow DSR

MORNING • AFTER BREAKFAST

Protects your stomach. It's a precaution because the pain medicine below can sometimes upset the stomach.

Etohix TH4

MORNING & NIGHT • AFTER FOOD

An anti-inflammatory plus a muscle relaxer. This is the main medicine for your back & neck pain.

Baga NT

NIGHT • AFTER DINNER

Calms the nerve signals that cause the tingling. Taken at night because it can make you drowsy — which helps you sleep.

Duloran 20 mg

MORNING & NIGHT • AFTER FOOD

Helps with long-term nerve pain. Also gently improves mood and sleep — useful since you mentioned stress & restless sleep.

WHAT'S NEXT

IN 1 WEEK

Follow-up visit on 30 June 2025 (Monday). Dr. Sharma will see how the medicines are working and review your blood test results.

SOON

Blood test results for vitamin B12, vitamin D, sugar (HbA1c), thyroid, liver, kidney, and a complete blood count. These tell us why the tingling started.

LIKELY

A pain-relief injection (epidural) at the lower-back level where the pressure is greatest. We'll decide the timing after your blood results & how the medicines are working.

ONGOING

Continue the medicines as listed. Don't skip doses — they work best when taken consistently. The full benefit takes 2-4 weeks to settle in.

When to reach out to us right away

DON'T WAIT FOR THE NEXT VISIT

Most days will be steady. But if any of these happen, call the hospital or come in immediately.

- Sudden weakness in your legs that wasn't there before
- Sudden severe headache, blurred vision, or trouble speaking
- Severe pain that doesn't respond to your usual medicines
- Loss of control over passing urine or stool
- Numbness that suddenly becomes much worse or spreads quickly
- Any chest discomfort, breathlessness, or fainting spells

You're doing the right things — coming in for the tests, taking the medicines, asking the questions. A clear plan is in place. We're here whenever you need us.